

Research Proposal

Private Equity Takeovers and Care for Medicaid-Dependent Nursing Homes

I'm interested in studying how private equity ownership affects nursing home care quality, with a particular focus on facilities that rely more heavily on Medicaid.

There's been a growing amount of policy discussion around PE in healthcare, especially in settings that serve vulnerable populations. Nursing homes are an important setting for this question because residents are often older, medically complex, and highly dependent on the quality of staffing and day-to-day care. At the same time, many facilities rely substantially on Medicaid reimbursement, which generally pays less than private-pay or Medicare sources. This creates an interesting and important policy question: when a private equity firm acquires a nursing home, are the effects different in places that are already more financially constrained because they serve a more Medicaid-dependent population? As such, my concise research question is:

Do private-equity takeovers of nursing homes have larger effects on quality and staffing outcomes in facilities that were more Medicaid-dependent before acquisition?

The motivation for this project comes from the currently relevant debate about whether private equity ownership changes incentives in ways that may affect patient care. Existing work has already looked at average effects of PE ownership on outcomes like staffing, mortality, and quality ratings. What I'm especially interested in is whether those effects are larger in facilities serving populations that are more reliant on public insurance.

From a health policy perspective, this matters because Medicaid-dependent nursing homes may be operating with thinner margins to begin with. If PE ownership leads to cost-cutting—particularly in labor-intensive inputs like nursing staff—the consequences could be especially meaningful for lower-income elderly populations and for public spending through Medicaid. Additionally, this topic allows me to explore a topic I'm passionate about researching in the future -

Medicaid cut impacts on low-income communities through proxies. This topic is similar to that, but in a way that I can study within the scope of this class.

A key part of this project is building the treatment sample in a transparent and reproducible way. Rather than relying only on proprietary deal databases, I'm identifying facility-level ownership transitions through publicly verifiable sources. These include CMS-linked ownership pages through ProPublica (already cleaned CMS data), older CMS-mirrored ownership records, public deal announcements, operator reports (both of prior from online sources), and some state-level reporting sources such as AARP Florida ownership-change documentation.

So far, I have identified **24 verified transitions from non-private-equity ownership to PE-backed ownership**. This currently includes:

- 17 Cascade / Legacy facilities in Iowa
- 3 Tryko / Marquis facilities in New Jersey
- 3 Gold FL Trust II facilities in Florida
- 1 Portopiccino-linked facility in Florida

Treatment has been conservatively defined. A facility is only included if the post-period ownership is clearly PE-backed in CMS-linked ownership data, the pre-period owner is visibly distinct and non-PE, and the timing of the transition can be tied to a plausible acquisition window. I'm excluding all ambiguous ownership restructurings and PE-to-PE transfers to ensure the study is very accurate.

Empirical

For outcomes, I'm planning to use publicly available nursing home measures, most likely from CMS Nursing Home Compare and related datasets. The main outcomes I'm interested in are:

- staffing levels
- inspection deficiencies
- quality ratings
- possibly hospitalization-related or other care-quality measures, depending on data availability

The key heterogeneity variable will be **pre-treatment Medicaid dependence**, likely measured over the one to two years before acquisition. My goal is to test whether PE takeovers have larger effects in facilities where Medicaid plays a larger role in the payer mix.

Empirically, I'm planning to use a **difference-in-differences framework**, and possibly an event-study specification if it makes sense to. Conceptually, the model would estimate the average effect of PE takeover while also interacting that effect with a measure of Medicaid dependence.

The main coefficient of interest is the interaction term, since that directly answers whether more Medicaid-dependent facilities experience larger changes after PE acquisition.

Feasibility

I think the project is feasible because the ownership-verification workflow is already underway, the treatment sample is partially built, and the core data sources are public and reproducible. My plan is to organize the final empirical dataset and all cleaning steps in reproducible Stata do-files.

One limitation is that this is not yet a national census of all PE nursing home acquisitions. The current sample is also somewhat geographically concentrated, especially in Iowa. Still, I think the transparency and verifiability of the ownership transitions is a strength, and I plan to continue expanding the sample. The study could even simply be limited to Iowa itself if needed, although I do think the effects will have good external validity.

Overall, my broader goal is to better understand whether PE ownership has differential effects on facilities serving more vulnerable, Medicaid-dependent populations, which I think has clear implications for health policy and healthcare equity.